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Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attn: CMS–0042–NC
7500 Security Boulevard
Baltimore, MD 21244-8013

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy and National Coordinator for Health Information Technology
Office of the Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology (ASTP/ONC)
Department of Health and Human Services
330 C St SW
Washington, DC 20201

Submitted electronically via: http://www.regulations.gov

RE: CMS-0042-NC - Request for Information: Health Technology Ecosystem [RIN 0938–AV68]

Dear Drs. Oz and Keane:

Oscar Health, Inc. ("Oscar," "we," "us," "our") appreciates the opportunity to provide information in response to the Centers for Medicare & Medicaid Services ("CMS") Request for Information; Health Technology Ecosystem [RIN 0938–AV68] (the "RFI"). Oscar is a leading health insurance and healthcare technology company focused exclusively on the individual market. Powered by a proprietary full-stack technology platform, Oscar offers a suite of services designed to build member trust, leverage personalized data, and ensure access to quality, affordable care.

We share the Department of Health and Human Services' ("HHS'") interest in building a patient-centered health system, and applaud HHS for seeking public comment on the digital health market and interoperability. As a leading health technology company and Qualified Health Plan ("QHP") Issuer, Oscar is subject to many of the same Health Information Technology ("Health IT") requirements as Medicare plans. This experience gives us unique insight into both the transformative potential of technology in healthcare and the key barriers to improving the broader health technology infrastructure.

Oscar is a strong supporter of achieving true interoperability in healthcare and encouraging broader use of Health IT, but we continue to encounter several barriers to realizing this vision. To address these barriers, Oscar encourages CMS to take the following actions:

- Spur broader adoption and utilization of Fast Healthcare Interoperability Resources ("FHIR")-based Application Programming Interface ("API") technologies by encouraging competition and innovation among developers and exploring improvements for FHIR standards.

- Explore the Trusted Exchange Framework and Common Agreement ("TEFCA") and non-TEFCA solutions to strengthen health information exchange across the health system.
- Stimulate broad acceptance of digital identity credentials by creating incentives for adoption and continuing to build public trust.
- Create a national provider directory that imports state-specific requirements, streamlines the credentialing process, and requires up-to-date information.
- Explore tools to enhance the bulk extraction of clinical quality data from electronic health information ("EHI"), including through the use of AI.

In addition, Oscar believes CMS should encourage broader adoption and use of AI to improve healthcare administration and patient experiences, both internally and in the private sector, and we welcome further discussion with CMS about our experiences with AI.

These issues and recommendations are discussed in more detail below.

1. FHIR-Based APIs (PA-2)

Oscar supports utilization of API-accessible technologies and has implemented CMS-required APIs. We believe API-accessible technologies can improve patient engagement, enhance care delivery, and reduce administrative burdens. For example, we believe electronic prior authorization allows for more automated approvals, which result in reduced administrative burden for providers and can improve clinical outcomes.

While we support the current FHIR-based API technologies, we believe certain aspects of their design and implementation remain cumbersome, which has restrained their adoption, utilization, and value. For example, the complexity and rigidity of existing FHIR-based API requirements have led Oscar to outsource these APIs, despite our preference to build technology in-house. Such outsourcing prevents us from building customized technology that prioritizes member experience and works well with ours and external systems. Furthermore, because FHIR-based APIs are so difficult to develop, there is limited market competition, innovation, and utilization.

CMS can accelerate implementation of FHIR-based APIs by encouraging greater competition and innovation among developers and exploring improvements for FHIR standards, which will result in improved access to EHI. For example, emerging technologies like model control protocols ("MCPs"), could be integrated with the FHIR-based APIs by embedding AI agents into the APIs—improving communication between systems and alleviating developer burden. Oscar would like to explore innovative solutions to improve these APIs and make them more accessible, but the burdens discussed above limit our ability to internally develop these APIs in accordance with current CMS standards. By reducing the burden of these technical standards and improving the quality of bulk data exports, more companies like Oscar could introduce more and better APIs, which would lead to greater adoption and improved interoperability and access to EHI. We would welcome the opportunity to discuss in greater detail the specific challenges we have encountered in implementing these standards, as well as potential improvements.

It is also critical that all payers are required to implement FHIR-based APIs and for providers to be incentivized, or required, to use FHIR-compatible software to realize the value of interoperability. In Oscar's experience, adoption and utilization of patient, provider, and payer-to-payer APIs is very low. Oscar has seen fewer than 100 calls on its provider access API, and even fewer on its patient access API. Instead, many providers rely on alternate, often obsolete, processes like fax and phone calls to receive timely patient information and make prior authorization requests. Furthermore, most private payers are not obligated to have systems that are compatible with the APIs, nor are they required to participate with

the Payer-to-Payer API, which will result in incomplete member and patient profiles when that API is live in 2027. Ensuring that all payers are required, and providers are incentivized or required, to use FHIR-based APIs or FHIR-compatible software would lead to more accurate and complete member and patient information.

2. TEFCA (PA-1)

Oscar supports HHS' interoperability goals and is enthusiastic to work with CMS to promote streamlined health information exchange among health care providers, payers, and patients. However, there may be approaches beyond TEFCA to meet this goal. While Oscar supports TEFCA's intent, in practice, it presents many barriers to data access. For example, TEFCA limits access to EHI unless the requester is accessing data for treatment purposes. As a result, payer access to necessary member records is significantly restricted unless the payer obtains members authorization, which can introduce additional administrative challenges (e.g., multiple forms, separate logins, and repeated consent requests). This may inadvertently limit access to critical patient information.

Additionally, Oscar is concerned that concentrating data access and exchange among a limited number of entities may create future barriers to data sharing. At present, there are only a small number of QHINs, many of which are already dominant in the health technology market. This concentration could potentially limit innovative approaches to health information exchange and discourage new entrants from participating in the market.

Finally, TEFCA's governance and policy agreements establish rigid legal frameworks and require adherence to technical standards that are not used by all healthcare organizations. These additional requirements, layered on top of existing federal and state laws, may discourage broader TEFCA adoption by payers. Oscar supports a nationwide network that is accessible to a wide range of players–including payers, providers, and tech-enabled health platforms. We encourage CMS and ASTP/ONC to fully engage all stakeholders in policy discussions to improve the exchange and access for EHI.

3. Digital Identity Credentials (PA-3)

Oscar supports a CMS initiative to encourage industry-wide acceptance of digital identity credentials. Because digital identity credentials are portable across platforms and organizations, they would significantly alleviate burdens for providers and members who currently maintain different login credentials across payers, facilities, providers, and other platforms. However, widespread adoption of digital identity credentials presents several challenges, particularly due to (i) limited industry adoption and therefore limited consumer and provider utilization; (ii) privacy, security, and data usage concerns among patients and companies alike; and (iii) potential consumer costs.

CMS should encourage digital identity credentials through incentives to increase industry-wide adoption, which we believe in turn will lead to greater utilization. CMS should also consider endorsing certain digital identity credentials, or developing a universal digital ID for healthcare or general use. Because of public concerns with privacy, security, data usage, and cost for consumers, however, we do not believe a CMS mandate would be appropriate at this time. We encourage CMS to continue to engage with health care stakeholders to build trust that will support industry-wide acceptance of digital identity credentials and, ultimately, greater patient utilization.

4. National Provider Directory (PA-4)

Oscar supports a national provider directory to serve as the primary database for providers and facilities to update their information, which payers would then access to update their network directories. We envision many benefits from a national provider directory, including allowing for more accurate provider information; alleviating administrative burden for providers, payers, and other third-parties; and providing current data to inform network adequacy and accuracy.

For example, a national provider directory could improve identification of workforce shortages, inform adjustments in network adequacy standards and exceptions review for payers, and reduce waste, fraud, and abuse. FHIR endpoints would allow for easier integration with other FHIR-based APIs required by CMS and maintained by certain payers, thereby improving interoperability. Further, a national database would alleviate administrative burdens for providers who currently must maintain their information across jurisdictions, licensing organizations, payers, and facilities—all of which use different login credentials and require different information.

However, there are challenges to provider directories today, which will also result in implementation issues for a national provider directory. First, providers are not presently required to update their data timely, which leads to inaccurate information on the directories. Second, provider directory information varies across all fifty states, so there are discrepancies with the categories and types of data that must be uploaded to the provider directories. Third, payers would be unable to directly sync their network directories with the national directory via API because payers may only list providers in their directories who have been credentialed. Payers would still have to manually credential each network provider in accordance with state law.

To achieve a national provider directory, we believe the required information for providers must be inclusive of state specific requirements and not just limited to federal requirements. Otherwise, the quality of the data will be incomplete and payers will still have to seek out alternate data sources and verification processes with providers to confirm the outstanding information. Additionally, including National Committee for Quality Assurance (NCQA) and URAC credentialing information in the required fields could allow payers to automate the credentialing process. This would reduce processing and posting time, and ultimately lead to more accurate provider directories to serve consumers—a consistent challenge across the industry.

Oscar is currently participating in the joint CMS-Oklahoma Insurance Department provider directory pilot, which will establish and launch an automated, one-stop shop, statewide, provider directory that will be accessible through the QHP Directory Pilot Portal.[1] Oscar would be happy to share our experiences from this pilot with CMS.

5. Clinical Quality Data (PA-5)

Oscar supports technology advances that enable bulk extraction of clinical quality data. Medical records are necessary for supporting our business and improving our members' experience and outcomes. For example, we use clinical quality data for processing claims, reviewing prior authorizations, and informing risk adjustment. Access to complete medical records for members would, among other things, (i) support compliance with the CMS Interoperability and Prior Authorization Final Rule (CMS-0057-F) and integration of the required data elements consistent with USCDI; (ii) allow Oscar to create care plans that are tailored a member's specific needs, thereby improving patient outcomes; and (iii) allow for more accurate risk profiles which informs rate development and risk adjustment data.

[1] https://www.cms.gov/files/document/qhp-directory-pilotfact-sheet.pdf

However, there are challenges with current bulk extraction of clinical quality data. One of the primary challenges we encounter with bulk data extraction from medical records is the inconsistency of EHI data across different sources. Not only do systems vary widely in how they structure and store information, but data is often presented in non-standard layouts that complicate automated extraction and analysis. This lack of uniformity can result in incomplete or inaccurate data transfers, increased administrative burden, and additional resources required to normalize and interpret the information.

To address these challenges, Oscar leverages the multimodal capabilities of AI models, enabling us to process not only text but also images and tables. We have found that by combining traditional optical character recognition ("OCR") and multimodal AI approaches, we can more accurately extract and interpret information from diverse medical record formats, improving data quality and utility for downstream applications. While these AI-driven solutions may benefit other payers facing similar challenges, we encourage CMS to also explore enhancements to the bulk extraction of clinical quality data, ensuring more consistent, standardized, and accessible data across the healthcare ecosystem.

6. Artificial Intelligence

Finally, while the RFI did not specifically request input from payers on the use of AI, we would like to highlight that Oscar leverages AI across our operations to enhance both business solutions and member experiences, in accordance with best practices and applicable law. We believe the most effective way to understand AI, including its risks and its potential, is through transparent experimentation, learning, and sharing. To that end, we have established robust internal governance processes to ensure our use of AI is responsible, transparent, and designed to safeguard against bias while building trust with our members. Further, recent advancements in AI offer significant opportunities to streamline many of the processes that make healthcare costly, complex, and time-consuming.

Oscar has used AI in several key areas to drive value for our members and providers. For example, we deploy AI to streamline contract audits and remediate billing discrepancies, helping to reduce fraud, waste, and abuse. We also use AI to analyze member calls with our Care Guides, allowing us to identify common pain points, proactively address frequent issues, and escalate calls when members express high levels of frustration or concern. Additionally, our Virtual Care team recently launched a generative AI "clinical intake bot" that collects symptom context from members before a provider joins, reducing administrative burden and allowing clinicians to focus more on patient care. We believe these efforts set us apart from other payers, and we would welcome further discussion with CMS and ASTP/ONC regarding HHS' AI strategy and policy.

Thank you for your time and consideration. Oscar welcomes the opportunity to participate in future ASTP/ONC and CMS initiatives to advance interoperability and digital health innovation. We look forward to continued collaboration to achieve a more connected, patient-centered healthcare system. Please do not hesitate to contact Oscar's Director of Public Policy, Stephanie Akpa, at sakpa@hioscar.com if you have any questions or would like additional information.

Sincerely,

Mark Bertolini
Chief Executive Officer
Oscar Health